

D11 — IR Consult

Dr. F. Alvarez, Interventional Radiology | Interventional Radiology | 02/22/2026 06:00

Patient: Pemberton, Harold J. **MRN:** 847-29-3156 **DOB:** 04/12/1951 **Allergies:** Sulfa drugs (rash)

INTERVENTIONAL RADIOLOGY CONSULT NOTE

Patient: Harold J. Pemberton | MRN: 847-29-3156
Date: 02/22/2026 06:00 | Provider: Dr. F. Alvarez, Interventional Radiology
Reason for Consult: Recurrent upper GI hemorrhage, management options

CLINICAL SUMMARY:
74 y/o male with recurrent upper GI bleeding from gastric ulcer. Initial endoscopic hemostasis with clips on 02/21 failed within 24 hours. Currently hemodynamically unstable with Hgb 6.2, HR 118, SBP 88.

ASSESSMENT:
Endoscopic hemostasis has failed. Clips placed less than 24 hours ago did not maintain hemostasis. In our experience, repeat endoscopic intervention after early clip failure carries a significant risk of re-failure and may delay definitive treatment. Additionally, repeat endoscopy with cautery in a patient on systemic corticosteroids and with recent clip failure raises concern for perforation risk.

RECOMMENDATION:
Angiographic embolization of the left gastric artery. Technical success rate for angiographic embolization of gastric bleeding is >90%. This offers more durable hemostasis than repeat endoscopy in the setting of clip failure.

Hold anticoagulation minimum 5 days post-embolization. Reassess for restart no earlier than Day 10 of hospitalization (02/27/2026).

If the medicine team elects to proceed, we can schedule the procedure for this afternoon.

Electronically signed by: Dr. F. Alvarez, MD
Date: 02/22/2026 06:30